



POLICE DEPARTMENT

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In the Matter of the Disciplinary Proceedings :

- against - :

Police Officer Joseph Bailey :

Tax Registry No. 952437 :

Military & Extended Leave Desk :

FINAL

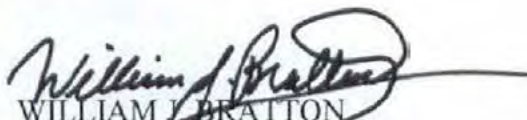
ORDER

OF

DISMISSAL
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Police Officer Joseph Bailey, Tax Registry No. 952437, Shield No. 21778, Social Security No. ending in [REDACTED], having been served with written notice, has been tried on written Charges and Specifications numbered 2014-12459, as set forth on form P.D. 468-121, dated September 26, 2014, and after a review of the entire record, has been found Guilty as Charged.

Now therefore, pursuant to the powers vested in me by Section 14-115 of the Administrative Code of the City of New York, I hereby DISMISS Police Officer Joseph Bailey from the Police Service of the City of New York.


WILLIAM J. BRATTON
POLICE COMMISSIONER

EFFECTIVE: 0001 hrs. September 22, 2015



POLICE DEPARTMENT

September 9, 2015

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In the Matter of the Charges and Specifications : Case No.
- against - : 2014-12459
Police Officer Joseph Bailey :
Tax Registry No. 952437 :
Military & Extended Leave Desk :
-----X

At: Police Headquarters
One Police Plaza
New York, New York 10038

Before: Honorable Rosemarie Maldonado
Deputy Commissioner - Trials

APPEARANCE:

For the Department:

David Green, Esq.
Department Advocate's Office
One Police Plaza
New York, New York 10038

For the Respondent:

Harvey A. Levine
Levine & Gilbert
115 Christopher Street
New York, New York 10014

To:

HONORABLE WILLIAM J. BRATTON
POLICE COMMISSIONER
ONE POLICE PLAZA
NEW YORK, NEW YORK 10038

The above-named member of the Department appeared before me on June 22, 2015, charged with the following:

1. Said Police Officer Joseph Bailey, while assigned to the Patrol Borough Queens South IRT or the 107th Precinct, on or about and between March 15, 2014 and September 15, 2014, engaged in conduct prejudicial to the good order, efficiency, or discipline of the Department, in that said Police Officer wrongfully ingested cocaine.

P.G. 203-10, Page 1, Paragraph 5 – PUBLIC CONTACT - PROHIBITED
CONDUCT - GENERAL REGULATIONS

2. Said Police Officer Joseph Bailey, while assigned to the Patrol Borough Queens South IRT or the 107th Precinct, on or about and between March 15, 2014 and September 15, 2014, engaged in conduct prejudicial to the good order, efficiency, or discipline of the Department, in that said Police Officer wrongfully possessed cocaine.

P.G. 203-10, Page 1, Paragraph 5 – PUBLIC CONTACT - PROHIBITED
CONDUCT - GENERAL REGULATIONS

The Department was represented by David Green, Esq., Department Advocate's Office, and Respondent was represented by Harvey A. Levine, Esq. Respondent, through his counsel, entered a plea of Not Guilty to the subject charges. The Department called Dr. Thomas Cairns as a witness. Respondent called Sheila Alayo-Bailey and Roger Bailey Jr. as witnesses and testified on his own behalf. A stenographic transcript of the trial record has been prepared and is available for the Police Commissioner's review.

DECISION

After evaluating the testimony and evidence presented at the hearing, this tribunal finds Respondent Guilty of the subject charges.

SUMMARY OF EVIDENCE PRESENTED

The following facts are undisputed. On September 15, 2014, at 0728 hours, Respondent received notification to report for a random drug test. At 0836 hours, he signed in at the Medical Division. Police Officer Sandro Scacchi, assigned to the Medical Division's Drug Screening Unit, met with Respondent and explained the collection procedure. Respondent then completed the Drug Screening Questionnaire Forms and the Psychemedics Forensic Drug Testing Custody and Control Forms. As required, each form was signed by Respondent and Officer Scacchi. (Court Ex. 1).

Officer Scacchi collected hair samples from Respondent's leg because the length of his head hair did not meet testing standards.¹ Officer Scacchi then divided the hair into three equal samples and placed each in a separate sealed envelope containing a copy of the custody and control forms signed by Respondent. The hair samples were secured in a locker and remained there until two samples were sent to Psychemedics Corporation (Psychemedics), a drug testing laboratory in Culver City, California.²

Psychemedics Results

Dr. Thomas Cairns is the Senior Scientific Advisor and Deputy Lab Director at Psychemedics. Psychemedics has been certified by the Food and Drug Administration (FDA) and is licensed by the New York State Department of Health to practice forensic toxicology. Dr. Cairns himself is also licensed by the New York Department of Health to practice forensic toxicology. Based on the qualifications cited in his curriculum vitae, he

¹ It is undisputed that a urine sample was also collected from Respondent on September 15, 2014, and that the test result was negative.

² Respondent did not contest the sample collection process or the integrity of the sample's chain of custody.

was deemed an expert in the field of forensic toxicology, specifically as it relates to hair testing analysis. (Tr. 18-19, 22-23; Dept. Exs. 1-2).

Dr. Cairns explained that when cocaine is ingested into the body, it is absorbed into the bloodstream. As cocaine-contaminated blood passes through the liver, it is metabolized into its primary metabolite benzoylecgonine (BE). Cocaine and BE are carried by the bloodstream to the base of every strand of hair. As cocaine-contaminated blood enters the base of a strand of hair, both cocaine and BE get trapped in the follicle and are incorporated into its structure. According to Dr. Cairns, a 2.5 cm long hair sample taken from the leg records each drug use over the course of approximately six to seven months. (Tr. 30-31, 45-46).

The first test performed by Psychemedics on a hair sample is enzyme immunoassay (EIA). If this screening analysis finds a presence of cocaine at a concentration level at or above the administrative cutoff level of 5 nanograms per 10 milligrams of hair (5ng/10mg), it is considered presumptive positive. In presumptive positive cases, another portion of the sample is aggressively washed to remove possible external contamination. Dr. Cairns explained that the aggressive wash procedure is composed of six steps and takes approximately three and a half hours to complete. After the wash procedure is completed, the sample is then analyzed using liquid chromatography, mass spectrometry, mass spectrometry (LC/MS/MS). If LC/MS/MS analysis finds a presence of cocaine at or above the administrative cutoff level, the lab will test another hair sample for confirmation. Dr. Cairns explained that the cutoff level is established by the FDA and supported by clinical analysis to clearly differentiate

someone who has ingested cocaine on multiple occasions from someone who has used it once or has been subjected to "micro" exposure. (Tr. 33-43).

At trial, Dr. Cairns reviewed the laboratory data package produced by Psychemedics for the samples collected from Respondent on September 15, 2014. The package, in which Respondent was identified by his donor identification number, 02-3464-14-XH, showed that the chain of custody remained intact. Respondent's first LC/MS/MS analysis indicated the presence of cocaine at a level of 13.3ng/10mg, more than double the cutoff of 5ng/10mg. The first sample also tested positive for the presence of the metabolite BE at a level of 1.18ng/10mg, which constitutes a concentration of 8.8% of the cocaine detected.³ Respondent's second sample confirmed the presence of cocaine at a level of 15.2ng/10mg, more than three times the cutoff. The second sample also tested positive for the presence of the metabolite BE at a level of 1.12ng/10mg, which constitutes a concentration of 7.3% of the cocaine detected.⁴ According to Dr. Cairns, these results represent multiple ingestions of cocaine over the look back period. (Tr. 42-46, 57-59; Dept. Ex. 3).

The day after Respondent was notified that the hair samples collected by the Medical Division had tested positive for cocaine, he went to Omega Laboratories and had another drug test performed using leg hair as samples. According to the Omega Result Report dated October 1, 2014, Respondent tested positive for cocaine at a level of 14.3ng/10mg. Dr. Cairns testified that the positive results from Omega validate the

³ Dr. Cairns explained that the federal government requires that the presence of BE be at a concentration level at or above five percent of the cocaine detected in order for a sample to test positive.

⁴ Dr. Cairns explained that the difference in the two results was not significant because the range of hair lengths collected from Respondent's leg varied.

accuracy of the original two hair tests performed by Psychomedics. (Tr. 60-63, 118-119; Dept. Ex. 5).

FINDINGS AND ANALYSIS

Respondent does not contest that his random drug test revealed the presence of cocaine and the cocaine metabolite BE within his system. In fact, Respondent himself introduced into evidence an independent drug test he undertook confirming those results. Instead, Respondent denies having ever used cocaine and claims that his positive test results were caused by unknowing exposure to his father's use of crack cocaine in the family's shared bathroom. Respondent further contends that Psychomedics' decontamination procedures failed to eliminate the risk of false positives caused by this type of concentrated environmental exposure. After evaluating the testimony and evidence presented at the hearing, and assessing the credibility of the witnesses, this tribunal finds that the preponderance of the credible evidence established that Respondent is guilty of possessing and ingesting cocaine without police necessity. (Dept. Ex. 5).

Respondent, his father and his mother appeared at trial and testified in his defense. At the hearing, Respondent's father, Roger Bailey, Jr., admitted that he has been addicted to crack cocaine since 1987. He described himself as a "functional" user but admitted that he has been institutionalized twice in an effort to overcome his dependency. Bailey testified that recently it has become a habit to smoke crack cocaine in the shared family bathroom for at least four hours a night, three to five times a week. Bailey claims that he smoked crack cocaine when Respondent worked the midnight shift and the rest of his family was in bed. He attempted to hide his cocaine usage by purposely putting a

towel under the bathroom door and window to prevent any smoke from escaping. Each time he ingested the drug he left the bathroom "heavily smoked" and covered with "a thick brownish or yellowish residue." Afterwards, Bailey or his wife cleaned the bathroom thoroughly in an effort to remove the residue from the walls, cabinets and other surfaces. Bailey asserts that his son has never smoked cigarettes or marijuana and does not even know what crack cocaine looks like. (Tr. 162-163, 166-169, 173-174).

Respondent's mother, Sheila Alayo-Bailey, testified that her husband has been struggling with addiction to crack cocaine since before Respondent was born. Despite her protestations, and while Respondent was at work, Bailey smoked crack cocaine in the family's upstairs bathroom. She confirmed that when he exited that room there was a "cloud of smoke" and a "thick yellow" residue throughout. She cleaned all the bathroom surfaces within her reach and aired the room out in an attempt to remove any trace of her husband's drug use. She did this in part to prevent Respondent from knowing that his father had relapsed. Although she did her best, there were portions of the bathroom on which residue remained. Alayo-Bailey added that she "know[s] for a fact that [her] son," has never used cocaine. She described him as a "homebody" who is not interested in going out to bars or clubs and has never even smoked a cigarette. (Tr. 140, 146-147, 152-153, 157).

At the hearing, Respondent denied ever ingesting cocaine. He lives with his mother, father, and younger brother in a three bedroom, two bathroom house. Since childhood he has been aware of his father's addiction. Respondent has smelled cocaine in his home, has seen his father under the influence of drugs, and has experienced the negative impact addiction has had on his family. Respondent asserted that he believed

his father had stopped smoking crack because he was unemployed and did not have money to purchase the narcotic. Respondent testified that he did not know his father had resumed smoking crack and explained that was why he had not disclosed this information to the Department. (Tr. 108-113, 125-128).

At trial, Respondent asserted that prior to May 2014, he resided in the basement of his parent's house and used a private bathroom. Respondent claims that sometime in May 2014, he moved upstairs with the rest of his family because the basement environment aggravated his allergies. At that time he also began to regularly use the family bathroom. He kept a toothbrush in the bathroom medicine cabinet and a hair brush and razor blade on a bathroom shelf. (Tr. 109-110, 115-116).

Respondent challenged the wash procedure Psychmedics uses for hair samples and argued that it does not rule out the possibility of a sample testing positive as a result of involuntary external contamination. In support of this argument, Respondent's counsel described the enormity of use of crack cocaine smoked by Respondent's father in their enclosed bathroom and the likelihood that its ensuing residue contaminated Respondent's hair sample. In support of this argument, Respondent's counsel cited to a 2013 Massachusetts Civil Service Commission decision rejecting certain Psychmedics test results, in part, due to questions about its decontamination process. (Tr. 72, 80-81).

In support of his defense of involuntary ingestion, Respondent points to his three previous Department drug tests which indicated negative results for cocaine. It is his contention that during the time period those tests were performed, he was living in the basement of his parents' house and was not sharing a bathroom. Respondent argued that

it was only after he moved upstairs and began sharing the family bathroom in which his father smoked crack that he tested positive for cocaine. (Tr. 114-118, 125-127, 129)

It is important to note that Respondent only presented the testimony of his parents to confirm that factual underpinnings of his defense. His parents, however, cannot be considered unbiased witnesses. More importantly, Respondent did not present independent scientific evidence to support his explanation for the alleged circumstances under which he claims to have involuntarily ingested this narcotic. In contrast, the Department's witness, Dr. Cairns, convincingly testified at trial that Respondent's test results for cocaine were elevated to a level that reflected multiple ingestions during the look back period. The Department also satisfied its burden of proving that Psychemedics' wash procedure and administrative cutoff levels prevent hair samples from resulting in false positives under the circumstances described by Respondent in this case.

Dr. Cairns explained that external contamination occurs when a drug is deposited on the outside of the hair. When hair is contaminated externally, neither the drug nor its metabolites gets trapped in the hair structure, as it does when the drug is ingested and enters the bloodstream. Dr. Cairns testified that even in cases where leg hair is exposed to active smoke or powder cocaine, which did not occur in the case at hand, it is sufficiently removed by the aggressive wash procedure used by Psychemedics. While Dr. Cairns did admit that there is a possibility that the wash procedure may not remove all traces of external contamination from a hair sample, the use of the administrative cutoff further ensures that only individuals who have actually ingested the drug on

multiple occasions test positive. Therefore, any trace levels remaining on the hair samples would be too low to cause a sample to reach the cutoff. (Tr. 32, 36-37, 48).

Dr. Cairns cited an August 2014 peer-reviewed study, commonly referred to as "The FBI Study," which determined that the aggressive wash procedures employed by Psychomedics are sufficient to avoid false positives.⁵ Dr. Cairns explained that the FBI had been using hair analysis to test for drug abuse, but became "uncomfortable that they were unable to differentiate ingestion from external exposure." In fact, the 2013 Massachusetts Civil Service Commission decision cited by Respondent was based, in part, on the FBI's concerns and discontinuance of the test. To resolve the issue, the FBI sent two of its own chemists to the Psychomedics laboratory in California to learn how to perform the aggressive wash procedure. The FBI chemists then returned to their own lab and conducted their own research study. The chemists concluded that the wash procedures used by Psychomedics were sufficient to "exclude false-positive results from environmental contact" with cocaine.⁶ (Tr. 49-50; Dept. Ex. 4).

Moreover, Dr. Cairns convincingly discredited each of Respondent's proffered methods of external contamination as incapable of causing a positive result at or above the administrative cutoff. Respondent's counsel first argued that because Respondent's father smoked crack cocaine in the bathroom without any kind of ventilation, it is possible that when Respondent later used the bathroom, he was inhaling lingering smoke and crack cocaine residue left on the solid surfaces of the bathroom. Dr. Cairns first explained that the vapors exhaled by the crack cocaine user are the pyrolysis product of

⁵ Cynthia L. Morris-Kukoski et al., *Analysis of Extensively Washed Hair from Cocaine Users and Drug Chemists to Establish New Reporting Criteria*, 38 J. Anal. Toxicol. 628 (November/December 2014).

⁶ *Id.*

crack cocaine, not crack cocaine itself, and contain "exceptionally low levels" of cocaine. Dr. Cairns further explained that the inhalation of secondhand smoke is known as micro ingestion. Since Respondent was never in the bathroom with his father while he was actually smoking crack cocaine, no micro ingestion occurred. Furthermore, any remaining vapors are absorbed onto solid surfaces and transformed into a tarry substance. Once those vapors are absorbed onto the solid surfaces of a bathroom, they are no longer "available in the atmosphere to breathe in." According to Dr. Cairns, even if someone were to scrape the tarry pyrolysis residue off of the walls or other surfaces, he or she would have to consume gallons of it each day to even reach trace levels in their system. (Tr. 70-72, 77, 97-99).

That was not the case here. In fact, Respondent's parents stated that they aired out and cleaned the bathroom to remove residue on the surfaces. By doing so it is likely that they removed the crack cocaine pyrolysis residue further minimizing the risk of micro ingestion. Dr. Cairns supported his conclusion by citing to a study published in the Journal of Clinical Pharmacology in which scientists exposed hair to an amount of crack cocaine smoke equivalent to 5,000 lines of cocaine and then performed an aggressive wash on the hair samples. None of the samples in that study tested positive for cocaine. (Tr. 90, 147-148, 168-169).

Respondent also presented a scenario in which an individual could be exposed to cocaine through an open wound. According to Dr. Cairns, nanogram levels of cocaine could enter the bloodstream through contact with the pyrolysis residue in the bathroom and an open wound. However, once again, the levels of exposure would be so low that a recorded level above the cutoff would not be possible. According to Dr. Cairns, "you

would have to inhale lines and lines and lines and lines of cocaine powder to even come close to [the] cutoff," of 5ng/10mg. On cross-examination, Respondent testified that he has had open wounds, but could not be more specific. He also admitted that he never intentionally rubbed an open wound on a bathroom surface. Thus, it is unlikely that he tested positive as a result of this type of open wound exposure. (Tr. 76-77, 98-99, 101, 124-125).

Respondent also argued that his toothbrush was exposed to crack cocaine smoke in the bathroom. Respondent testified that although the toothbrush was kept in the bathroom's medicine cabinet, the door to the cabinet did not close completely. As a result, Respondent argued that he was brushing his teeth daily with a toothbrush that contained a buildup of the crack cocaine pyrolysis residue. Respondent provided no scientific evidence to support this claim of contamination. Dr. Cairns explained that while it is possible for cocaine to enter the bloodstream through the mucus membranes of the mouth, any residue on Respondent's toothbrush would again, be too low to cause Respondent to test at or above the cutoff. (Tr. 69-71, 103-104, 149-150; Resp. Ex. A).

Respondent's unsubstantiated arguments regarding the reliability of Psychomedics testing procedures and external contamination that occurred in the bathroom referred to as a "den of iniquity," were unpersuasive and lacked scientific support. Even crediting the testimony at trial that Respondent's father smoked crack cocaine in the family bathroom for extended periods, the scientific evidence presented supports a conclusion that Respondent could not have tested positive for cocaine at levels of 13.3ng/10mg and 15.2ng/10mg simply by sharing that bathroom with his father. (Tr. 90, 183).

Accordingly, I find that the Department Advocate met its burden of proof as to the reliability of the Psychedics testing procedures and those results stand. *See Matter of McBride v. Kelly*, 215 A.D.2d 161 (1st Dept. 1995) (substantial evidence that officer ingested cocaine was provided by mass spectrometry). Thus, Respondent is found Guilty of the charged misconduct.

PENALTY

In order to determine an appropriate penalty, Respondent's service record was examined. *See Matter of Pell v. Board of Education*, 34 N.Y.2d 222 (1974). Respondent was appointed to the Department on July 11, 2012. Information from his personnel record that was considered in making this penalty recommendation is contained in an attached confidential memorandum.

Respondent has been found Guilty of wrongfully possessing and ingesting cocaine without authority or police necessity. The Department has a strong interest in not employing individuals who ingest and possess illegal drugs such as cocaine. Accordingly, the Court recommends that the Respondent be DISMISSED from the New York City Police Department. *See Disciplinary Case No. 2010-3064*, signed May 8, 2012 (13-year member with no prior disciplinary history dismissed from the Department for possessing and ingesting cocaine); *Disciplinary Case No. 2010-86365*, signed January 17, 2012 (19-year detective with no prior disciplinary record dismissed from the Department for ingesting and possessing cocaine), *aff'd sub nom Matter of Jones v. Kelly* (2013 NY Slip Op 07262); *Disciplinary Case No. 2013-9490*, signed June 16, 2015 (13-

year member with no prior disciplinary history record dismissed from the Department for possessing and ingesting cocaine).

Respectfully submitted,



Rosemarie Maldonado
Deputy Commissioner Trials

APPROVED

SEP 22 2015

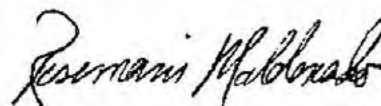
WILLIAM J. BRATTON
POLICE COMMISSIONER

POLICE DEPARTMENT
CITY OF NEW YORK

From: Deputy Commissioner Trials
To: Police Commissioner
Subject: CONFIDENTIAL MEMORANDUM
POLICE OFFICER JOSEPH BAILEY
TAX REGISTRY NO. 952437
DISCIPLINARY CASE NO. 2014-12459

Respondent received an overall rating of 3.5 on his most recent performance evaluation, and 3.0, on his two prior performance evaluations. [REDACTED]
[REDACTED] He has been suspended from duty since September 25, 2014, as a result of the charges and specifications in this case. He has no prior formal disciplinary record.

For your consideration.



Rosemarie Maldonado
Deputy Commissioner Trials